BASELINE → TREATMENT → FOLLOW-UP

Checks across both phases

Relative stages, not equally spaced dates. Ask who books and reviews each check. Do not enter dates to interpret this guide.

Check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Cycle 2+ · Day 1	Cycle 2+ · Day 22	Completion / follow-up
Blood counts, kidney and liver	Baseline full blood count (FBC) and renal/liver profile	Review before treatment	Review before second/final chemotherapy	Review before every Day 1 dose	Review before every Day 22 dose	Continue unresolved-toxicity checks per plan
Glucose, thyroid and immune symptoms	Glucose, thyroid function tests (TFTs), morning cortisol; hepatitis B surface antigen (HBsAg)/hepatitis C virus (HCV) RNA	Glucose/thyroid function tests (TFT) before this 42-day cycle; immune symptom review	Immune symptom review; extra tests if indicated	Glucose/TFT before every 42-day cycle; symptom review	Immune symptom review; extra tests if indicated	Symptoms can occur at least five months after last dose
Vitamins and steroids	Folate lead-in; B12 in preceding week; reconcile steroid plan	Daily folate; rash/antiemetic medicines as prescribed	Final planned pemetrexed: repeat prescribed rash/ antiemetic course	No further pemetrexed: complete 21-day folate tail; review other support	No automatic B12/rash steroids or induction antiemetics	Agree stop dates and ongoing medicines; no abrupt steroid taper
Platinum precautions	Renal, hearing and nerve review; hydration/electrolyte plan	Review hydration, electrolytes and renal function	Review counts, kidney, hearing and nerves again	Platinum has stopped; review persisting effects	Platinum remains stopped	Follow any residual kidney, hearing or nerve effects

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00714 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab, ipilimumab, pemetrexed and cisplatin: treatment and checks

First-line metastatic non-small-cell lung cancer (NSCLC) · confirm pemetrexed-compatible histology

The first cycle lasts 42 days: all four medicines on Day 1, then nivolumab, pemetrexed and cisplatin on Day 22. Chemotherapy ends after those two visits. From Cycle 2, nivolumab continues on Days 1 and 22 and ipilimumab on Day 1 of each 42-day cycle. Immune treatment continues while appropriate until the cancer progresses, side effects require stopping, or up to 24 months without progression. Plans can change after review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Cycle 2+ · Day 1	Cycle 2+ · Day 22	Completion / follow-up
Treatment NCCP 00714	Confirm indication, histology and plan	All four medicines	Nivolumab + pemetrexed + cisplatin; NO ipilimumab	Nivolumab + ipilimumab; NO chemotherapy	Nivolumab only; NO ipilimumab or chemotherapy	Stop at progression/toxicity or up to 24 months; agree follow-up
Blood counts, kidney and liver NCCP 00714	Baseline FBC and renal/liver profile	Review before treatment	Review before second/final chemotherapy	Review before every Day 1 dose	Review before every Day 22 dose	Continue unresolved-toxicity checks per plan
Glucose, thyroid and immune symptoms NCCP 00714	Glucose, TFTs, morning cortisol; HBsAg/HCV RNA	Glucose/TFT before this 42-day cycle; immune symptom review	Immune symptom review; extra tests if indicated	Glucose/TFT before every 42-day cycle; symptom review	Immune symptom review; extra tests if indicated	Symptoms can occur at least five months after last dose
Vitamins and steroids NCCP 00714	Folate lead-in; B12 in preceding week; reconcile steroid plan	Daily folate; rash/antiemetic medicines as prescribed	Final planned pemetrexed: repeat prescribed rash/ antiemetic course	No further pemetrexed: complete 21-day folate tail; review other support	No automatic B12/rash steroids or induction antiemetics	Agree stop dates and ongoing medicines; no abrupt steroid taper
Platinum precautions NCCP 00714	Renal, hearing and nerve review; hydration/electrolyte plan	Review hydration, electrolytes and renal function	Review counts, kidney, hearing and nerves again	Platinum has stopped; review persisting effects	Platinum remains stopped	Follow any residual kidney, hearing or nerve effects

What each check is for

1 Baseline and first treatment

Before Cycle 1 Day 1

Confirm suitability and preventive medicines

Ask / **act:** Confirm results, prescriptions, contacts and the actual chemotherapy backbone.

Sources: NCCP 00714

2 Final chemotherapy visit

Cycle 1 Day 22 (21 days after first visit)

Two chemotherapy administrations complete the planned induction

Ask / **act:** Check next phase and stop/review chemotherapy support; continue folate for 21 days after final pemetrexed.

Sources: NCCP 00714

3 Immune continuation

Cycle 2 onwards, Days 1 and 22

Different medicines and support at alternating visits

Ask / **act:** Review joint immune hold rules and duration; maintain alert-card education.

Sources: NCCP 00714

4 End of treatment

At progression, unacceptable toxicity or up to 24 months without progression

Delayed immune effects and unresolved toxicities still need care

Ask / **act:** Provide responsible-team, monitoring and urgent-contact arrangements.

Sources: NCCP 00714

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.27